

MARKET LANDSCAPE • COMPETITIVE SCAN

AI-Native Companies in Healthcare Claims

Compiled June 2026 • US-centric • AI-first companies (legacy RCM incumbents — Optum, R1, Waystar — excluded). Funding/clients/positioning are point-in-time.

Managed services? **Yes** = runs the process (managed / BPaaS) • **Hybrid** = software + managed option • **No** = software the client operates. Clients shown are publicly disclosed; "not public" = customers exist but undisclosed.

★ = most relevant to an intake / referral + payer-call use case.

1 • Claims intake, referral & payer-call automation

- ★ **Tennr** — AI referral/intake front office: document intake + voice AI auto-calling payers for benefits, follow-up, confirmations. ~10M docs/mo; \$101M Series C (~\$605M val).
Managed services: **No** — software/orchestration platform. Clients: DME suppliers, home-health agencies, infusion centers, specialty pharmacies, imaging centers.
- ★ **Infinitus** — voice AI for payer-facing work: eligibility, prior auth, claim status, EOB retrieval, appeals, phone submissions. 150+ data points/call.
Managed services: **No** — voice-AI automation platform. Clients: health systems, pharma, payers (names not public).

2 • Autonomous medical coding (charts → claims)

- **Fathom** — autonomous coding for high-volume specialties (ED, radiology, primary care, surgery). #1 "Reducing Cost of Care," 2025 KLAS.
Managed services: **Yes** — coding delivered as an outcome (~90% of charts). Clients: health systems & large physician groups (names largely not public).
- **CodaMetrix** — autonomous coding for hospitals/health systems; CMX CARE uses the longitudinal record for context.
Managed services: **Yes** — SaaS-delivered autonomous coding. Clients: 30+ US health systems incl. Mass General Brigham (~58.7% fewer radiology coding denials).
- **Nym Health** — semantic autonomous coder; strong ED product; Epic Showroom "Fully Autonomous Coding."
Managed services: **Yes** — autonomous coding engine. Clients: 21 providers incl. Geisinger and Ochsner Health.
- **Arintra** — charts → claims at ~96% accuracy; ~43% fewer coding denials, ~11% less undercoding.
Managed services: **Yes** — autonomous (zero-touch) coding. Clients: health systems & large physician groups (names not public).
- **RapidClaims • MediCodio • Maverick Medical AI** — newer AI-native coding/claims-automation entrants.
Managed services: **Hybrid** — coding software with services options. Clients: provider groups / RCM teams (names not public).

3 • Provider RCM, claims automation & denials

- **Candid Health** — full claims-lifecycle automation; >95% touchless claims reported.
Managed services: **No** — RCM software platform. Clients: 200+ orgs, weighted to digital-health & specialty practices.
- **Adonis** — AI orchestration + agents for RCM/denials; \$40M Series C ('26), >\$95M total.
Managed services: **No** — software/agents the client's team runs. Clients: Baptist Health South Florida, ApolloMD, Bicycle Health, Fox Valley Orthopedics; ~20,000 providers.
- **AKASA** — generative AI across the revenue cycle, trained on each system's own data.
Managed services: **No** — software for in-house RCM staff. Clients: Cleveland Clinic; Oracle/Cerner partnership.
- **Sohar Health** — API-first front-end RCM (eligibility + claim accuracy), ~95% automation.
Managed services: **No** — API/software. Clients: digital-health & behavioral-health providers (names largely not public).
- **Syntra • Taiga • LunaBill** — billing/claims for private practices; LunaBill = voice AI for billing follow-up calls.
Managed services: **Yes** (Syntra/Taiga run billing for practices); **No** (LunaBill = tool). Clients: independent/private physician practices.
- **Aegis** — automates insurance denial appeals end-to-end for providers.
Managed services: **Yes** — runs the appeal process (intake → resolution). Clients: healthcare providers (names not public).

4 • Prior authorization

- **Cohere Health** — market leader; ~47M payer-provider interactions/yr, 85% real-time approvals; ~\$200M raised.
Managed services: **Hybrid** — in-house software + delegated end-to-end managed (Cohere Complete™). Clients: Humana (national MA + Commercial), Geisinger Health Plan, CMS; ~660K providers.
- **Anterior** (fmr. Co:Helm) — AI for health plans: prior auth, payment integrity, risk adjustment; \$40M raised.
Managed services: **No / unconfirmed** — payer back-office automation. Clients: Geisinger Health Plan (others not public).

5 • Payer-side: payment integrity & claims adjudication

- **Machinify** — unified pre-pay + post-pay payment-integrity platform across the claim lifecycle.
Managed services: **Hybrid** — platform + recovery services (model not fully public). Clients: national/regional health plans (names not public).
- **Anomaly (Anomaly Insights)** — AI payer-intelligence for denials/underpayments/adjudication deviations; +\$17M ('26).
Managed services: **No** — software/intelligence platform. Clients: 20+ health systems, diagnostic labs, outsourced-RCM firms.

- **Alaffia Health** — AI + expert clinicians for end-to-end claims ops (UM, payment integrity, appeals); \$55M Series B (Feb '26).
Managed services: **Yes** — both software subscriptions and managed services; runs bill/chart reviews for plans. Clients: Medicaid, Medicare Advantage & commercial plans (≤2M members), TPAs, cost-containment firms, gov agencies.
- **Curacel · Amera** — AI claims processing/automation for payers & TPAs; Amera structures messy PDF/EDI/non-standard inputs.
Managed services: **No** — claims infrastructure/software. Clients: payers, TPAs, providers (Curacel strong in emerging markets); names not public.

6 • Consumer / patient-side denial appeals

- **Counterforce Health** — free AI appeal-letter generator; ~70% appeal success; copies state regulators on filings.
Managed services: **Yes** (performed for the patient; free). Clients: individual patients/consumers.
- **Claimable** — AI appeals (~\$40/letter); deals with drugmakers & health systems to appeal on patients' behalf.
Managed services: **Yes** (appeals run on the patient's behalf). Clients: individual patients; drugmaker & health-system partnerships.

Sources

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Internal competitive-landscape reference. Figures point-in-time as of June 2026; software-vs-service positioning shifts as companies move up-market.